



Dak Guru

A Self Learning Portal

www.dakguru.com

CS (MA) Rules, 1944

DISCLAIMER: *This booklet has been prepared strictly from an examination point of view. While every effort has been made to ensure accuracy and the inclusion of recent changes, amendments, and updates to the relevant Acts, Rules, and Regulations, this document is intended as a study aid only. Readers and students are strongly recommended to refer to official government sources or gazettes for the most reliable and authoritative information.*

CS (Medical Attendance) Rules, 1944

Rule 1. Short Title and extent of Application.

(1) These rules may be called the Central Services (Medical Attendance) Rules, 1944. Notification Dated 01 January, 1944.

(2) These Rules apply to-

(a) all Government servants except under point (3), whose conditions of service are prescribed by Rules made or deemed to be made by the Central Government, when they are on duty leave or Foreign Service in India or when under suspension. (b) All Govt. servants who are on leave preparatory to retirement. (c) Govt. pensioners on their re-employment. (d) Defense Accounts Department personnel. (e) Civilian paid from defense services estimates. (f) Central Govt. on terminal leave. (g) A probationer. (h) Apprentices who are in the whole-time service of Govt.. (i) All state Govt. servants on deputation with the Central Govt..

(3) These rules do not apply to-

(a) Those in railway service. (b) Defense services personnel. (c) Government Servants who are on deputation abroad. (d) those of non-Gazetted rank stationed in or passing through Calcutta. (e) Retired Govt. officials. (f) Non-officials sent on deputation abroad. (g) Govt. servants who are governed by the CGHS scheme while in stations where this scheme is functioning. (h) Officer of All India Service. (i) India based officers and staff serving in Mission abroad.

Government Decisions

(1) All Govt. Servants and their families, they are governed under these rules, while on tour or on leave visiting any place, where CGHS is in force. They can avail CGHS facilities. (2) Claims in respect of officials admitted before retirement but discharged after superannuation the expenses should be reimbursed by official's own office.

Rule 2. Definition

(a) Authorized Medical Attendant (AMA)- Authorized Medical Attendant (AMA) is Medical Officer in the employee of Central Government or Private Medical Practitioner appointed/nominated by the Ministry/Department for providing medical attendance to its employees. (b) Patient -a government servant to whom these Rules apply and who has fallen ill. (c) "Treatment" means the use of all medical and surgical facilities available at the Government hospital in which the Govt. servant is treated.

Government Decisions

(1) Appointment of private registered medical as AMAs by the Head of Department. (2) No treatment from AMA while he is on regular leave. (3) Consultation/ visiting/ injection fee of AMA

I. Medical Post Graduate/ Specialists

- First consultation: Rs. 100/-
- Subsequent Consultation: Rs. 60/-

II. Medical Graduate (MBBS or equivalent)

- First consultation: Rs. 70/-
- Subsequent Consultation: Rs. 40/-

III. Injection Fees

- Intramuscular/subcutaneous: Rs. 20/-
- Intravenous: Rs. 40/-

(4) Simultaneous treatment in two or more systems of medicine for other ailments. (5) Reimbursement of charges for clinical test paid to private practitioners/Institution.

- I. Up to **Rs. 500/-** a certificate from the medical officer in charge of the case in the Hospital that such facilities are not available in Govt. Hospital.

- II. Exceeding Rs. 500/- The prior approval of the Medical Superintendent of the Hospital.

(6) Ceiling for reimbursement of special Nursing and Ayah/Attendant

- Special Nurse: Rs. 150/- per shift of **12 hours**.
- Ayah/Attendant: Rs. 75/- per shift of **12 hours**.

(7) The age of woman undergoing IVF (In-vitro Fertilization) treatment procedure should be between **21 and 39 years**. The woman has to be married and living with her husband. The reimbursement of expenditure incurred on IVF procedure will be allowed up to maximum of **3 (three)** fresh cycles. An amount not exceeding Rs. 65000/- per cycle or the actual cost, whichever is lower. (8) Beneficiaries availing tests/investigations at empaneled Pvt. Hospitals on the recommendation of AMA, the medical prescription issued by AMA shall be treated for a single use within a period of **30 days** from the date of prescription. The medical prescription would require revalidation after expiry of validity period. Prescription/advice issued by AMA for any procedure is valid for **three months**. (9) Stay in hospital: - The average stay in hospital as in-patient should be reckoned as **two weeks**. Any stay beyond **three weeks** should be considered as prolonged stay.

Rule 3. Medical Attendance

(1) A Government servant shall be entitled, free of charge to medical attendance by the authorised medical attendant; (2) Where a Government servant is entitled under sub-rule (1), free of charge, to receive medical attendance, any amount paid by him on account of such medical attendance shall, on production of a certificate in writing by the authorized medical attendant in this behalf be reimbursed to him by the Central Government. Note: If any conditions the controlling authority reject the claim, the claimant may submit an appeal to the appropriate authority within a period of **Forty-five days** of the date of receipt of the order rejecting the claim.

Government Decisions

(a) First **four consultations/visits** should be completed within a period of **ten days** from the date of commencement of treatment in respect of one single and continuous spell of

illness in respect of a particular disease. (b) In Indian system of Medicines and Homoeopathy the **10 days** limit may be increased up to **20 days** by AMA. (c) Maximum **15 (Fifteen) injections** administered in case of treatment of non-prolonged nature of treatment at consulting room of the AMA.

Rule 4. Traveling Allowance for Medical Attendance Journeys

(1) When the place at which a patient falls ill is more than **five miles (8 KM)** by the shortest route from the consulting room of the authorized medical attendant-

(a) the patient shall be entitled to traveling allowance for the journey to and from such consulting room, or (b) if the patient is too ill to travel the authorized medical attendant shall be entitled to traveling allowance for the journey to and from the place where the patients

(2) Application for traveling allowance under sub-rule (i) shall be accompanied by a certificate in writing by the authorized medical attendant stating that medical attendance was necessary and if the application is under clause (b) of that sub-rule that the patient was too ill to travel.

Travelling Allowance

(1) The attendant will be entitled to travelling allowance both ways at the rate admissible under these orders to a member of family of the Govt. servant. (2) Kidney donor-

(a) Donor is private person -T.A. will be admissible at the rates applicable to the recipient Govt. Servant. (b) Donor is another Govt. Servant or his family member- T.A. will be admissible at the rates applicable to the donor Govt. servant.

(3) Special concession to TB and Cancer Patients. The concession for admission to or discharge from a hospital/clinic in connection with their re-examination or periodical check-up-

S.1. no.	For Whom available	Nature of concession
1.	Patient travelling accompanied by an attendant	Patient and attendant 1/2 normal journey fare

2.	Patient travelling alone	Single Journey ticket on 1/4th the normal fare due
----	--------------------------	--

Rule 5. Consultation with Specialist

(1) If the authorized medical attendant is of opinion that the case of a patient is of such a serious or special nature as to require medical attendance by some person other than himself, he may, with the approval of the Chief Administrative Medical Officer of the State (which shall be obtained beforehand unless the delay involved entails danger to the health of the patient)

(a) send the patient to the nearest specialist or other medical officer, by whom, in his opinion, medical attendance is required for the patient; or

(b) if the patient is too ill to travel, summon such specialist or other medical officer to attend upon the patient.

(2) A patient sent under clause (a) of sub-rule (1) shall on production of a certificate in writing by the authorised medical journeys to and from the headquarters of the specialist or other medical officer.

(3) A specialist or other medical officer summoned under clause (b) of subrule (1) shall, on production of a certificate in writing by the authorised medical attendant in this behalf be entitled to travelling allowance for the journey to and from the place where the patient is.

Government Decisions

(a) Testing of eyesight for glasses at recognized hospitals by specialists- Once in every **three years** on the recommendations of the AMA. Fee paid to the specialist will be reimbursed.

Rule 6. Medical Treatment

(1) A Government servant shall be entitled, free of charge, to treatment-

(a) in such Government hospital at or near the place where he falls ill as can in the opinion of the authorised medical attendant provide the necessary and suitable treatment; or

(b) if there is no such hospital as is referred to in sub-clause (a) in such hospital other than a government hospital at or near the place as can in the opinion of the authorised medical attendant, provide the necessary and suitable treatment;

(2) Where a Government servant is entitled under sub-rule (1), free of charge, to treatment in a hospital, any amount paid by him on account of such treatment shall, on production of a certificate in writing by the authorised medical attendant in this behalf, be reimbursed to him by the Central Government.

Note: If any conditions the controlling authority reject the claim, the claimant may submit an appeal to the appropriate authority within a period of **Forty-five days** of the date of receipt of the order rejecting the claim.

(a) Prolonged treatment- Either required treatment more than **4 consultations/visits** within a period of **10 days** from the date of commencement of treatment, or more than **15 injections**, as the case may be. A patient should be referred to OPD.

(b) Treatment for Immunizing and prophylactic purposes-On account of treatment Immunizing and Prophylactic purposes should be refundable to Govt. Servants in respect of treatment for self or family members in the case of communicable disease only, viz. Cholera, Typhoid group of fevers (TAB), Plague, Diphtheria, Whooping cough, tetanus and Poliomyelitis.

(c) Vaccination for Hepatitis B reimbursable.

Rule 7. Treatment at Residence

(1) If the authorised medical attendant is of opinion that owing to the absence or remoteness of a suitable hospital or to the severity of the illness, a Govt. servant cannot be given treatment as provided in clause (a) of sub-rule (1) of Rule 6, the Government servant may receive treatment at his residence.

(2) A Government servant receiving treatment at his residence under sub-rule (1) shall be entitled to receive towards the cost of such treatment incurred by him a sum equivalent of the cost of such treatment as he would have been entitled, free of charge, to receive under these rules if he had not been treated at his residence.

(3) Claims for sums admissible under sub-rule (2) shall be accompanied by a certificate in writing by the authorised medical attendant stating-

(a) his reasons for the opinion referred to in sub-rule (1); and

(b) the cost of similar treatment referred to in sub-rule (2).

Note: If the AMA certifies that Govt. servant required hospital treatment but that no accommodation was available at the recognized Hospital, then the fees paid for medical treatment at the patient's residence may be reimbursed.

Rule 8. Other Medical Facilities

(1) Charges for services rendered in connection with but not included in medical attendance on, or treatment of, a patient entitled, free of charge, to medical attendance or treatment under these Rules, shall be determined by the authorised medical attendant and paid by the patient.

(2) If any question arises as to whether any service is included in medical attendance or treatment it shall be referred to the Government and the decision of the Government shall be final.

Note: -

(a) Reimbursement of expenditure incurred on account of treatment of sterility will be admissible.

(b) Expenses incurred in connection with an operation for sterilization are refundable.

(c) Expenses incurred by the Central Govt. servants on medical termination of pregnancy is reimbursable, provided the medical termination of pregnancy has been performed at Govt. or other institutions/hospitals approved under the Medical Termination of Pregnancy Act, 1971.

(d) Reimbursement of complete denture-

- Complete denture- up to **Rs. 2000/-**

- Denture of one Jaw- **Rs. 1000/-**
- (e) Reimbursement of cost of various artificial appliances of purchase/repair/replacement. Maintenance cost will be borne by the beneficiary.
- (f) The appliances will be allowed for re-issue on completion of **5 years** in case of adults and **2 years** in the case of children except motorized wheel chair and tricycle.
- (g) Motorized wheel chair and tricycle will be re-issued after **5 years**.
- (h) Prosthetic components and Orthotic joints used in appliances should have BIS/CE (European) certification for the purposes of reimbursement and fabricated by firms having qualified prosthetist/orthotists.
- (i) Up to age **12 years** considered as physical growth age.
- (j) Any appliance that is not included in annexures. In the case where item cost is below **Rs. 50000/-** can be purchased with three quotations with the permission of HOD and where cost is more than **Rs. 50000/-** can purchased with prior permission of Additional Directors, CGHS of the concerned city or concerned DDG. On the basis of three quotations and approval of Technical Standing committee.
- (k) The list of items and rates will be revised every **5 years**.
- (l) The ceiling limit for reimbursement of expenditure incurred on purchase of a digital hearing aid for one ear only. Replacement of hearing aid may permit after **5 years** on the basis of condemnation certificate issued by the technical expert on approval of CGHS/Govt. ENT surgeon.
- Body worn/Pocket type: **Rs. 3000/-**
 - Analogue BTE: **Rs. 7000/-**
 - Digital BTE: **Rs. 8000/-**
 - Digital ITC/CIC: **Rs. 9000/-**

 **LEGISLATIVE NOTE:** The limits for Digital BTE and Digital ITC/CIC were substituted for "Rs. 15000/-" and "Rs. 20000/-" respectively, vide OM No. Z.11011/37/2019-EHS and reaffirmed in subsequent guidelines. (Updated 2025).

(m) Ceiling rates for intra-ocular lenses (IOL)

- (a) Hydrophobic Foldable IOL - **Rs. 5800/-**
- (b) Hydrophobic Acrylic IOL - **Rs. 4500/-**
- (c) PMMA IOL (AC/PC) - **Rs. 500/-**
- (d) Scleral Fixated IOL - **Rs. 3500/-**

(n) Ceiling limit for reimbursement of the cost of Coronary Stents. **Rs. 60000/-** per stents to a maximum of **two stents** in a patient or the actual cost, whichever is less.

(o) Ceiling rates for reimbursement of Bariatric surgery **Rs. 225000/-** or actual whichever is less.

(p) The ceiling rate for reimbursement for cochlear implant shall be **Rs. 535000/-**

(q) Entitlement for Indoor treatment: -

Basic Pay	Ward Entitlement
Up to Rs. 47600	General
Rs. 47601 to 63100	Semi Pvt. Ward
Rs. 63101 & above	Private Ward

(r) Annual Medical Examination for the group 'A' officer of Central civil services of age **40 years** and above-

- Men officers: **Rs. 2000/-** or actual whichever is less.
- Women Officer: **Rs. 2200/-** or actual whichever is less.

Rule 9. Countersignature of Certificates

The Controlling Officer of a patient may require that any certificate required by these Rules to be given by the authorised medical attendant for traveling allowance purposes shall be countersigned-

(a) in the case of a certificate given by the principal medical officer of a district, by the Chief Administrative Medical Officer of the State; and (b) in the case of a certificate given by any other medical officer, by the principal medical officer of the district

Rule 10. Transfer to Foreign Service

No Government servant shall be transferred to foreign service unless the foreign employer undertakes to afford to him so far as may be privileges not inferior to those which he would have enjoyed under these Rules if he had been employed in the service of the Government of India.

Rule 11. Treatment Outside India

(1) A Government Servant shall be eligible to obtain medical treatment outside India or, as the case may be, to claim reimbursement of the cost of medical treatment obtained inside or outside India in accordance with provisions of this rule. (2) A Government Servant desirous of availing of medical treatment outside India may make an application through the Department/Ministry to which the Government servant is attached to the Standing Committee established under this rule in the form specified by the Standing Committee. (3) A Government servant desiring to avail of medical treatment outside India for himself or for a member of his family for any treatment specified in the Table below shall, subject to the other provisions of this rule, be eligible for medical treatment outside India

Table

(1) Complex/high risk Cardio Vascular Surgery cases for treatment at Centers with extensive experience; (2) Bone marrow Transplant; (3) Complex Medical and Oncological Disorders, such as leukemia and Neo-plastic conditions; (4) Complex high risk cases in Micro Vascular and Neuro Surgery for treatment at Centers with extensive experience; (5) Treatment of extremely complex ailments other than those mentioned above which in

the opinion of Standing Committee can only be treated abroad and fall in the high risk category

(4) It shall be competent for the Central Government to review from time to time the list of treatment facilities as specified in the Table to sub-rule (3) and make such additions or deletions as it may deem fit by notification in the Official Gazette. (5) The Central Government may for purposes of this rule, constitute a Standing Committee consisting of :-

(a) the Director-General of Health Services in the Ministry of Health in the Central Government, (b) the Director-General of Armed Forces Medical Services. (c) the Director-General of the Indian Council of Medical Research, and (d) the Joint Secretary in the Ministry of Health and Family Welfare (Convener), for purposes of considering and recommending to the Central Government cases for medical treatment outside India.

(6) On receipt of an application for medical treatment outside India, the Standing Committee may, it after due consideration, satisfied that the ailment or treatment can be treated only outside India, issue a certificate to the concerned Department or Ministry to which the applicant Government servant is attached conveying its approval of the application and the concerned Department or Ministry shall, on the strength of that certificate incur necessary expenditure in getting the Government servant concerned or the member of his family treated outside India in accordance with the procedure laid down by the Standing Committee (7) It shall be competent for the Central Government to authorize reimbursement of expenditure on medical treatment obtained outside India, if it is satisfied that the prior approval could not be obtained by the Central Government servant due to circumstances beyond control; Provided that the Government servant fulfils all other conditions relating to medical treatment outside India under this rule. (8) The Standing Committee may, if it is satisfied that in the interest of the Government servant or the member of his family obtaining treatment abroad it is essential so to do, recommend **one attendant** to accompany the Government servant or the member of his family, as the case may be, and the expenditure so incurred shall also be eligible for reimbursement. (9) Where the Standing Committee, on receipt of an application for medical treatment outside India consider that adequate facility for treatment of the ailment sought to be treatment is available in any medical institution within India, it shall record such a finding and authorize treatment of such ailment in such medical institution

within India whereupon the cost of such treatment shall be reimbursed. (10) For purposes of sub-rule (9), the Ministry of Health in consultation with the Standing Committee shall, from time to time, notify the names of such institutions along with the ailments and the types of treatment available in such institutions. (11) The scale of expenditure and the eligibility for treatment for which a Government servant or a member of his family shall be entitled, shall be identical to the scale of expenditure and the eligibility of an official of the Indian Foreign Service of the corresponding grade in the Ministry of external Affairs under any Assisted Medical Attendance Scheme for the time being in force.

Govt. Decisions

(1) Duplicate cash memos in case of loss duly countersigned by AMA. (2) Cancellation of cash memoranda attached to medical claims. (3) Designation and degree of the doctor to be indicated on claim papers. (4) Time limit for medical claims submission.

- Complete of treatment out door patient - **6 months**.
- Discharged from Hospital (In-patient) - **6 months**.
- In case of Advance from discharged - **1 Month**.

 **LEGISLATIVE NOTE:** Substituted for "3 months" vide O.M. No. S.14025/19/2015-MS/CGHS(P) and subsequent orders standardizing the time limit to **6 months** for all outpatient claims. (Updated 2025).

(5) Approval limit of Medical Reimbursement in emergency there is no financial limit and up to **Rs. 2 lakhs** Head of office not below rank of Joint secretary and above Rs. 2 lakh IFA.

2. Grant of Advance

(1) Ordinarily, not more than **one advance** should be granted in respect of the same illness or injury. (2) Advance limit (a) Rs. 10000/- indoor treatment in hospital and out-patient treatment for illness like cancer, etc., where the duration of treatment is **3 months** or less. (b) **80%** of estimated cost or Rs. 36000/- whichever is less Advance granted for TB where the duration of treatment is more than **3 months**. (c) **90%** of packed deal, whenever is exists, for major illness of Bypass surgery, Kidney transplant, major cancer treatment, etc.

(3) The advance will be paid direct to the hospital concerned on receipt of an estimate from the treating Physician/Medical Superintendent of the Hospital.

3. Treatment of Special Diseases

(1) The duration of treatment can be extended for a maximum period of **three years** in mental diseases. (2) Treatment should be allowed initially for **six months**. Further extendable in blocks of **six months** in metal diseases. (3) Duration of treatment- The maximum period of treatment which may be allowed from the date of the detection of the disease for the purposes of the reimbursement should be restricted to **24 months** as certified by the TB specialist/ Medical Attendant. (4) Cancer: - Treatment can be availed in any recognized cancer institutes on the recommendation of AMA. (5) Post treatment check-up suggested is also admissible without recommendation of AMA. (6) Diabetes: - Cost of medicines may be reimbursed even for a period beyond **three months** on the advice of AMA.

4. Concession for Families

(1) The family of a Govt. servant is entitled to receive medical attendance and treatment at the Hospital at which Govt. servant himself is entitled. (2) Family means for these rules- wife or husband, parents, widowed sisters, widowed daughters, minor brothers, children, step children separated/ divorced daughters, and stepmother wholly dependent upon the Govt. servant and are normally residing with the Govt. Servant and other than station of Government of servant. (3) Age limit of dependents.

S.L. NO.	Members	Age
1	Son/ adopted legal child	Till he starts earning or attains the age of 25 years or gets married, whichever is earlier.
2	Daughter	Till she starts earning or gets married, irrespective of the age limit, whichever is earlier.
3	Son suffering from any permanent disability or any kind (Physical & mental)	Irrespective of age limit.

4	Dependent divorced/abandoned or separated from their husband/widowed daughters and dependent unmarried/divorced/abandoned or separated from their husband/widowed sisters.	Irrespective of age limit.
5	Minor brother (s)	Up to the age becoming a major.
6	Permanently disabled brothers	Irrespective of age limit.

(4) Woman's hospitals for male children up to **6 years**. (5) A member of the family is treated as dependent only if his/her income from all sources including pension and family pension is less than Rs. 9,000/- plus DA thereon.

- (a) The term 'children' will include children adopted legally.
- (b) The term 'wife' includes more than one wife.
- (c) Parents may reside either with the Govt. servant or with the family members at another station.

(6) When both husband and wife are Govt. Servants both can claim reimbursement of medical expenses for their respective parents. (7) A female Government employee can exercise an option to change to include her parents or her parent-in-law **once** during entire service. (8) Restriction of **two children** is not applicable for claiming medical reimbursement. (9) Whether a Government Employee who is under suspension is entitled for reimbursement all the medical expenditures involved at par with working employees. (10) Disability means: -

- a. Blindness
- b. low vision
- c. leprosy cured
- d. hearing impairment

- e. locomotor disability
- f. mental retardation
- g. mental illness

(11) Confinement, pre-natal and post-natal treatment -means treatment received by the wife of Govt. servant before and after child-birth for physiological or other disability attributable to child -birth.

CENTRAL GOVERNMENT HEALTH SCHEME (CGHS)

(1) Presently approximately **45 lakh** beneficiaries are covered by CGHS in **85 cities** all over India.

 **LEGISLATIVE NOTE:** Substituted for "38.5 lakh" and "74 cities" reflecting recent Ministry of Health and Family Welfare (MoHFW) expansion data.

(2) CGHS provides health care through following systems of Medicine-

- (1) Allopathic
- (2) Homoeopathic
- (3) Indian system of medicine
 - a. Ayurveda
 - b. Unani
 - c. Siddha and
 - d. Yoga

(3) Eligibility-

- (a) All Central Government employees paid from the Central Civil Estimates (except Railways and Delhi Administration), including their dependent family members residing in CGHS covered areas.

- (b) Pensioners of Central Government (except pensioners belonging to Railways and the Armed Forces) and their families.
- (c) Widows of Central Government pensioners, in receipt of family pension.

(4) For serving Central Government employees residing in CGHS covered area, obtaining a CGHS card is compulsory. The following deductions from salary of the employees are made by the department, every month, depending upon their pay in the pay matrix of 7th Pay commission (wef 1/1/2017) Corresponding levels in Pay Matrix as per 7th CPC Contribution per month:

Level in Pay Matrix	Contribution per month
Level 1-5	Rs. 250
Level 6	Rs. 450
Level 7-11	Rs. 650
Level 12 and above	Rs. 1000

Contribution to be made by the Pensioners/Family Pensioners would be the amount that they were subscribing at the time of their retirement or at the time of the death of government servant. Pensioners who want to avail CGHS facilities can make contribution either on **yearly basis** or **one time (ten years)** contribution for whole life validity.

Corresponding Basic pay drawn by the officer in 7th CPC per month for Ward Entitlement:

Ward Entitlement	Basic Pay
1 General Ward	Up to Rs. 47,600/-
2 Semi Private Ward	Rs. 47,601 to 63100/-
3 Private Ward	Rs. 63101 and above.

(5) Card color scheme- CGHS Plastic cards have the following colour at the top:

- Serving Government employees - **Blue**
- Pensioners, ex-MPs, Freedom Fighters, etc. - **Green**
- Member of Parliament - **Red**

- Beneficiaries of Autonomous Bodies / Journalist - **Yellow**.

(6) Plastic Duplicate CGHS card can be obtained against lost card by applying for the same to the concerned Additional Director along with a copy of FIR and will be charged Rs. 50/-. (7) Fixed medical allowance to central Government pensioners/Family pensioners residing in area not covered under CGHS is Rs. 1000/- per month. (8) A serving employee can apply for a pensioner CGHS Card along with his pension papers Application with enclosures and bank draft is to be forwarded to the CGHS through the office of the employee. The pensioner Card will be issued on the day of retirement (provided it is applied for at least **six weeks** before retirement date) and will get activated from next day. (9) Husband and wife both are also employed under central Government, they are not eligible for separate CGHS card. Spouse drawing higher pay shall make the card and others spouse is eligible to avail CGHS benefits as members. (10) The service card will be valid till the date of retirement if otherwise the employee is eligible. (11) In case of transfer of serving employee to a non CGHS covered area the service card will be valid for the family members up to **six months** after transfer provided CGHS contribution for **6 months** is made beforehand. (12) In case of serving CGHS beneficiary transferred from one department/ministry to another in the same city, the same CGHS card will continue. The new department/ministry will duly inform CGHS so that necessary changes can be made in the database of the employee. (13) Supply of Medicines from Wellness Centers

- (a) Medicines for chronic diseases may be issued for **3 months** at a time by any CGHS Doctor against the valid prescription of a specialist.
- (b) If the beneficiary is planning to go abroad, medicines prescribed may be issued up to **6 months** at a time with permission of Addl. Director for which an application with copy of valid prescription, CGHS Card and proof of travel has to be produced.

(14) Hospitalization in emergency

- (a) In emergency CGHS beneficiary may be admitted to any Govt/private hospital, but the reimbursement will be as per CGHS rates. However full reimbursement

may be allowed in specific cases on the recommendations of Technical Standing Committee (vide OM No. 4-18/2005-C&P-[Voll (Pt 1)] dated 20/2/2009).

- (b) In such cases, if reimbursement is required, the private hospital has to issue an emergency certificate stating in detail the condition of the patient at the time of admission which justified emergency.
- (c) CGHS empanelled hospitals cannot refuse admission in emergency to CGHS beneficiary and have to provide treatment on credit to pensioner beneficiaries and other entitled categories (even if the beneficiary is from outside CGHS city/other CGHS city).

(15) In case of treatment in emergency in non-empanelled hospital/expenditure incurred for treatment in empaneled hospital, Medical Reimbursement Claim (MRC) will have to be submitted by the beneficiary for reimbursement of expenses incurred. (16) The claim is to be submitted to the concerned department by serving employees and by the pensioner beneficiary within **6 months** of discharge the hospital.

 **LEGISLATIVE NOTE:** Substituted for "3 months" vide O.M. No. S.14025/19/2015-MS/CGHS(P) extending the time limit to **6 months** for medical claims submission. (Updated 2025).

(17) Empaneled Hospitals 1303 Empaneled Diagnostic Centres 206 as on **15.06.2020**. (18) Monetary ceiling for free diet: -The monetary ceiling for free diet for CGHS beneficiaries is revised to pay/pension / family pension of Rs. 44,900/- per month. (19) Monetary ceiling for free diet for beneficiaries suffering from TB or mental disease: The monetary ceiling for free diet in case of beneficiary suffering from TB or Mental disease is revised to pay / pension / family pension of Rs. 69,700/- per month. (20) Any pensioner / family pensioner who is entitled to avail CGHS facility, the rate of contribution shall be, he / she would have drawn in the post held by him/her (at the time of his / her retirement / death) had he / she continued to be in service now but for his/her retirement / death.